SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Weekly paclitaxel · relapsed or refractory SCLC

Lung · relapsed / refractory SCLC · NCCP 00226 · version 9 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Treatment aims to control small-cell lung cancer after previous therapy and may help symptoms. The evidence is limited, so discuss likely benefit, burdens and alternatives with the team.

Schedule
Paclitaxel 80 mg/m² is infused on Days 1, 8, 15 and 22 of a 28-day cycle. The consultant may instead prescribe Days 1, 8 and 15. Each planned weekly dose needs review. Continue while helping and tolerated; NCCP does not set a fixed cycle cap.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Paclitaxel

TAXANE CHEMOTHERAPY

- ☐ Low blood counts / infection
- ☐ Sensory or motor neuropathy, potentially persistent
- ☐ Hypersensitivity / infusion reaction
- ☐ Hair loss and nail / skin changes
- ☐ Muscle and joint aches
- ☐ Fatigue and mouth / bowel symptoms
- ☐ Rare serious lung or cardiac effects
- ☐ Serious infection / sepsis
- ☐ Rare serious bowel inflammation; vision changes

Discuss treatment aim, alternatives including supportive care, uncertainty, serious and lasting effects, fertility and contraception, premedication, IV access and the urgent contact plan. Explicitly document the off-label SCLC indication and weekly schedule under hospital policy; explain that the supporting study used a different weekly pattern.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00226 v9.

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